

1 _____ BILL NO. _____

2 INTRODUCED BY _____
3 (Primary Sponsor)

4 A BILL FOR AN ACT ENTITLED: "AN ACT ESTABLISHING A MEDICAL RESPITE CARE PROGRAM FOR
5 MEDICAID-ELIGIBLE HOMELESS INDIVIDUALS; PROVIDING RULEMAKING AUTHORITY; ESTABLISHING
6 REPORTING REQUIREMENTS; PROVIDING A DEFINITION; AND PROVIDING EFFECTIVE DATES."

7
8 BE IT ENACTED BY THE LEGISLATURE OF THE STATE OF MONTANA:

9
10 NEW SECTION. **Section 1. Medical respite care program for homeless individuals -- rulemaking**
11 **authority -- report.** (1) There is a medical respite care program for homeless individuals who are eligible for
12 medical assistance pursuant to Title 53, chapter 6.

13 (2) The department shall reimburse eligible providers for medical respite care that is offered in a
14 residential facility to individuals who are experiencing homelessness and who are too ill or frail to recover from
15 a physical illness or injury but are not ill enough to require hospital care.

16 (3) Medical respite care consists of short-term housing with supportive medical services for acute
17 and postacute care. To the extent allowable, medical respite care services include but are not limited to:

18 (a) development and monitoring of a treatment plan and administration of high-quality postacute
19 clinical services that are needed to help the patient meet the goals outlined in the treatment plan;

20 (b) medication management and medication reconciliation, including access to medications,
21 assistance with medication adherence, and medication education;

22 (c) immunizations;

23 (d) screening for communicable diseases;

24 (e) discharge planning and timelines for transitions into other settings;

25 (f) care coordination and referral management to ensure a patient is following up with medical
26 appointments as necessary;

27 (g) coordination with other levels of services;

28 (h) assistance with applying for and accessing social services programs;

- 1 (i) transportation related to hospital visits and medical appointments; and
2 (j) meals.
3 (4) The department may adopt rules to implement the provisions of this section.
4 (5) The department shall report annually by September 1 to the legislature in accordance with 5-
5 11-210 on:
6 (a) the number of homeless individuals served by each medical respite care facility;
7 (b) the cost of the program; and
8 (c) the reduction of health care costs due to the program's implementation.
9 (6) For the purposes of this section, "homeless" has the same meaning as provided in 42 U.S.C.
10 11302.
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12 NEW SECTION. Section 2. Direction to department of public health and human services. The
13 legislature directs the department of public health and human services to apply no later than January 1, 2026,
14 to the centers for medicare and medicaid services for a waiver or state plan amendment to allow for
15 implementation of the medical respite care program provided for in [section 1].
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17 NEW SECTION. Section 3. Codification instruction. [Section 1] is intended to be codified as an
18 integral part of Title 53, chapter 6, part 1, and the provisions of Title 53, chapter 6, part 1, apply to [section 1].
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20 NEW SECTION. Section 4. Effective date -- contingent effective date. (1) Except as provided in
21 subsection (2), [this act] is effective on passage and approval.

22 (2) [Section 1] is effective on the date that the director of the department of public health and
23 human services certifies to the code commissioner that the centers for medicare and medicaid services has
24 approved the department's application for a waiver or state plan amendment to implement the medical respite
25 care program. The director shall submit certification within 10 days after the occurrence of the contingency.
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